

Apollo Hospitals Chennai

21 Greams Lane, Off Greams Road, Chennai – 600006, Tamil Nadu

Ph: 044-28293333 | Email: records@apollochennai.com | PMJAY Empanelment: H001

PATIENT CLINICAL RECORD — INPATIENT DISCHARGE

Record No: APL/CHN/IPD/2025/010456 Date of Issue: 14/10/2025

SECTION 1: PATIENT AND ADMISSION INFORMATION

Patient Name:	Ramesh Kumar	IPD Number:	IPD/SG/2025/010456
Age / Gender:	28 Years / Male	PMJAY ID:	PMJAY-MH-2024-P004
Date of Birth:	15/03/1997	PMJAY Case ID:	IRIS-CASE1-20251010
Address:	45, MG Road, Bangalore – 560001	Contact:	+91-9876543210
Date of Admission:	10/10/2025 at 02:30 hrs	Date of Discharge:	14/10/2025 at 11:00 hrs
Duration of Stay:	4 Days	Type of Admission:	Emergency
Ward Category:	General Ward	PM-JAY Package Booked:	SG018A

SECTION 2: TREATING CONSULTANT

Consultant Name:	Dr. Suresh Patel	Registration No:	KMC-12345-2010
Qualification:	MBBS, MS (General Surgery)	Specialty:	General Surgery
Contact:	9845678901	Department:	General Surgery, Apollo Hospitals Chennai

SECTION 3: CLINICAL SUMMARY

Presenting Complaints:

Patient presented to emergency department with acute onset severe abdominal pain in right lower quadrant for past 12 hours. Pain started periumbilically and migrated to right iliac fossa. Associated with nausea, vomiting (3 episodes), and fever (101 F). Pain aggravated by movement and coughing.

Past Medical and Surgical History:

No significant past medical history. No chronic illnesses. No known drug allergies. No previous surgeries.

Pre-operative Examination and Investigations:

Physical Examination: Temperature 101.2 F, Pulse 110/min, BP 120/80 mmHg. Abdomen showed tenderness and guarding in right iliac fossa, positive McBurney's point tenderness, positive Rovsing's sign, and rebound tenderness. USG Abdomen (09/10/2025): Dilated appendix 9mm diameter with surrounding fluid collection suggestive of perforation and localized peritonitis. Blood Investigations (09/10/2025): TLC 18,500/cumm (elevated), Neutrophils 85%, CRP 45 mg/L (elevated). Serum electrolytes and renal function tests within normal limits. Blood sugar random 110 mg/dL.

SECTION 4: DIAGNOSIS

Provisional Diagnosis (at admission):	Acute Appendicitis with Perforation and Localized Peritonitis (ICD-10: K35.32)
Final Diagnosis (at discharge):	Post-operative status following emergency laparoscopic appendectomy with peritoneal lavage and drain placement for

SECTION 5: PROCEDURE PERFORMED

Procedure Name:	Emergency Laparoscopic Appendectomy with Peritoneal Lavage and Drain Placement
PM-JAY Package Code:	SG018A — Appendicular Perforation
Date and Time of Surgery:	10/10/2025 at 08:00 hrs
Duration:	75 minutes
Anaesthesia:	General Anaesthesia
Anaesthetist:	Dr. Priya Sharma

Operative Findings and Technique:

Standard 3-port laparoscopic technique used. On entry, perforated appendix identified with a localized abscess of approximately 30 mL purulent collection in the right iliac fossa. Appendectomy performed with endoloop ligation of appendicular base. Thorough

peritoneal lavage performed with warm normal saline until clear. A surgical drain placed in the right iliac fossa. No intraoperative complications. Estimated blood loss: 50 mL. No implants used.

SECTION 6: POST-OPERATIVE COURSE AND TREATMENT

Day 0 (10/10/2025): Post-op patient stable. IV antibiotics initiated — Inj. Ceftriaxone 1g BD and Inj. Metronidazole 500mg TDS. IV fluids maintained. NPO status. Vitals stable. Day 1 (11/10/2025): Patient afebrile. Started on oral fluids. Drain output sero-sanguinous, reducing. Ambulation encouraged. Day 2 (12/10/2025): Drain removed — output minimal and clear. Started on soft diet. Wound inspection — clean and dry. Day 3 (13/10/2025): Switched to oral antibiotics. Tolerating regular diet. No abdominal tenderness on examination. Day 4 (14/10/2025): Patient comfortable, afebrile, tolerating regular diet, all surgical wounds clean and dry, ambulatory without assistance. Declared medically fit for discharge.

SECTION 7: COMPLICATIONS

Intraoperative finding of perforated appendix with localized abscess (30 mL) — an unanticipated pathology that required modified surgical approach with peritoneal lavage and drain placement, and extended IV antibiotic coverage. No post-operative complications. Wound healing satisfactory.

SECTION 8: CONDITION AT DISCHARGE

Discharge Status:	Recovered	Discharge Condition:	Stable
Actual LOS:	4 Days (10/10/2025 to 14/10/2025)	Ward Category:	General Ward (Ward)
Bed Breakdown:	Ward: 4 days HDU: 0 ICU: 0 ICU-Vent: 0	Implants Used:	None
Complications:	None post-operative	Patient Amount Collected:	Rs. 11,500 (non-PM-JAY items)

SECTION 9: DISCHARGE MEDICATIONS

- Tab. Amoxicillin-Clavulanate 625mg — 1 tablet three times daily for 5 days
- Tab. Metronidazole 400mg — 1 tablet three times daily for 5 days
- Tab. Paracetamol 500mg — 1-2 tablets as needed for pain for 7 days
- Tab. Pantoprazole 40mg — 1 tablet once daily before breakfast for 7 days

SECTION 10: FOLLOW-UP INSTRUCTIONS

Follow-up Schedule: Day 7 post-surgery (17/10/2025) — wound check and suture removal at nearest PHC or surgical OPD. Day 14 post-surgery (24/10/2025) — final post-operative assessment.

Activity Restrictions: Avoid heavy lifting (more than 5 kg) for 2 weeks. Avoid strenuous exercise for 3 weeks.

Warning Signs — Return Immediately If: Fever above 100 F, severe abdominal pain, redness or discharge from wounds, or persistent vomiting.

PM-JAY Claim Reference: IRIS-CASE1-20251010 | Package: SG018A — Appendicular Perforation

SECTION 11: DOCUMENTS SUBMITTED WITH CLAIM

#	Document	Status
1	Discharge Summary (this document)	Available
2	Operative Notes — Laparoscopic Appendectomy 10/10/2025	Available
3	Pre-anaesthesia Check-up Report	Available
4	Blood Investigation Reports (CBC, CRP, RFT, Electrolytes)	Available
5	USG Abdomen Report (09/10/2025)	Available
6	Intraoperative Clinical Photograph	Available
7	Histopathology Report — Appendix Specimen	Pending (within 7 days)
8	Post-operative Wound Photograph	Available

SECTION 12: DECLARATIONS AND SIGNATURES

I certify that the patient was under my care, the treatment rendered was medically necessary, and all information provided in this record is true and correct to the best of my knowledge. The extended hospital stay was medically warranted due to intraoperative findings of perforated appendicitis with peritonitis requiring IV antibiotic therapy.

Dr. Suresh Patel MBBS, MS General Surgery Reg: KMC-12345-2010 Date: 14/10/2025	PMAM Signature Apollo Hospitals Chennai Date: 14/10/2025	Patient / Attendant Ramesh Kumar Date: 14/10/2025
--	---	--